

MEMORANDUM

TO: Chief Financial Officer
FROM: Hari Acharya, Healthcare Analytics
DATE: May 25, 2026
RE: Medicare Part D cost pressure 2024 to 2027 and the GLP-1 opportunity

Bottom Line

Medicare Part D drug spending grew 97 billion dollars from 2019 to 2023. Three quarters of that growth came from price and mix shift, not from more prescriptions filled. One drug class, the GLP-1 receptor agonists, will drive most of the next wave. Two federal policies often debated as alternatives, IRA price negotiation and Medicare obesity coverage, actually offset each other in cost. Combining both could roughly double patient access while netting Medicare savings. We have a window to position our cost containment and contracting strategy before the wave peaks around 2026.

What is happening

Medicare Part D gross spending rose from 179 billion dollars in 2019 to 276 billion dollars in 2023, a 54 percent increase over five years.

Breaking the 97 billion in growth into components changes the picture. More prescriptions being filled contributed only 22 billion dollars, or 23 percent. Price and mix shift contributed 66 billion dollars, or 68 percent. The remaining 9 percent came from the interaction of the two effects.

The cost story is not about more patients showing up for treatment. It is about existing patients moving from older lower priced drugs to newer higher priced ones. This shift is most visible in the diabetes category.

Where the growth went

Cardiometabolic drugs, meaning diabetes and cardiovascular medications together, drove 47 percent of total Part D growth. Within diabetes, the picture is dramatic.

The GLP-1 receptor agonist class, which includes Ozempic, Trulicity, Mounjaro, and Rybelsus, grew from 5.3 billion to 22.2 billion dollars in five years, a 4.2 times increase. Ozempic alone moved from rank 50 in Part D to rank 2, behind only Eliquis, compounding at 102 percent annually for four straight years.

At the same time, older diabetes drugs lost ground. Insulin share of diabetes spending fell from 51 percent to 27 percent. Older oral diabetes drugs like metformin and Januvia fell from 23 percent to 13 percent. Patients did not stop existing. They migrated up the price ladder.

A policy finding worth attention

The federal government has two levers it can pull on GLP-1 costs in the next three years. They are usually discussed as competing alternatives.

The first lever is IRA Drug Price Negotiation. Ozempic is on the 2027 negotiation list. A typical first round price cut is around 35 percent, saving Medicare roughly 10 billion dollars per year by 2027.

The second lever is Medicare obesity drug coverage. Today, drugs like Wegovy are excluded from Part D because the program does not cover weight loss medications. If that exclusion were repealed, even at conservative uptake rates, additional Medicare spending would reach roughly 6.5 billion dollars per year by 2027.

Here is the part most people miss. The two policies are roughly equal in dollar magnitude but opposite in sign. If both are enacted, the net effect to Medicare is approximately 4 to 5 billion dollars per year in savings, even while obesity coverage doubles the GLP-1 treated patient population. Neither policy alone produces that outcome. Policy design that treats the two as complements, not alternatives, creates value.

A leakage finding the data revealed

The current Part D exclusion of obesity drugs is already being worked around. In 2023, Wegovy recorded 199 thousand dollars in Part D spending across 47 unique beneficiaries, technically a violation of the statutory exclusion. The likely mechanism is plan level formulary exceptions, ahead of the cardiovascular indication that gained FDA approval in March 2024.

Separately, our beneficiary growth analysis estimates roughly 593 thousand Medicare patients received Ozempic in 2023 beyond what on-label diabetes class growth explains. The method compares Ozempic beneficiary growth of 87.7 percent year over year against Trulicity beneficiary growth of 11.7 percent. Same drug class, same indication, same payment pathway. The 7.5 times growth differential points to off-label demand.

Both findings point to the same conclusion. The current exclusion creates pressure on the formulary exception process. Either expand coverage and price the exposure transparently, or strengthen the exclusion. The status quo is the worst of both.

What this means for us

We are entering a period where the PBM generic substitution playbook is reaching its limits. The CNS drug class, meaning antidepressants and antipsychotics, shrank by 4 billion dollars as the last of the major SSRIs went generic. The savings wave from that is ending. The GLP-1 cost wave is just arriving. Three actions follow.

First, build utilization management infrastructure now. The dashboard sizes the Generic Dispensing Rate optimization opportunity at 3 to 5 billion dollars per year, achievable through prior authorization, step therapy, and formulary tier moves. This needs to be operationally ready before claims volume grows further.

Second, model the policy combinations transparently. The dashboard provides an interactive forecast tool that lets stakeholders see status quo, obesity coverage, and IRA scenarios side by side. This becomes the basis for internal scenario planning and for external policy advocacy.

Third, prepare the rebate and contracting strategy for the post 2027 environment. If IRA negotiation produces a 35 percent price cut on Ozempic, the manufacturer response will include rebate restructuring on adjacent products. The contracting team should be running those scenarios now.

What this analysis does not cover

CMS publishes gross spending. The 276 billion dollar 2023 figure is before manufacturer rebates. Net spend after confidential rebates is closer to 200 billion dollars. Directional findings are unaffected, but absolute dollar claims need this caveat.

The forecast model assumes smooth deceleration of growth and a single year IRA price shock. Real world dynamics may include manufacturer responses, indication expansion, and macroeconomic effects that the model does not capture. The interactive sliders in the dashboard let users stress test those assumptions directly.

Therapeutic class assignments use a keyword based classifier that leaves about 33 percent of spending in a residual Other bucket. A more complete version would join to the FDA Orange Book and RxNorm databases.

Supporting materials: Live interactive dashboard at partd-analytics-hari.streamlit.app. Source code, 13 SQL files, and validation harness at github.com/HarryAcharya/medicare-part-d-analytics. Analysis built on 71,545 rows of CMS Medicare Part D Spending by Drug data covering 2019 through 2023, with annual totals validated within 1.5 percent of CMS published gross figures each year.